WHAT IS CYBERCHONDRIA?

It is a clinical phenomenon in which curiosity about an ailment can lead to anxiety, fuelled by the ubiquity of Internet search engines and AI tools. Patients notice a symptom and do a quick online search which spirals into repeated, obsessive checking

DOC'S ORDERS

DOs

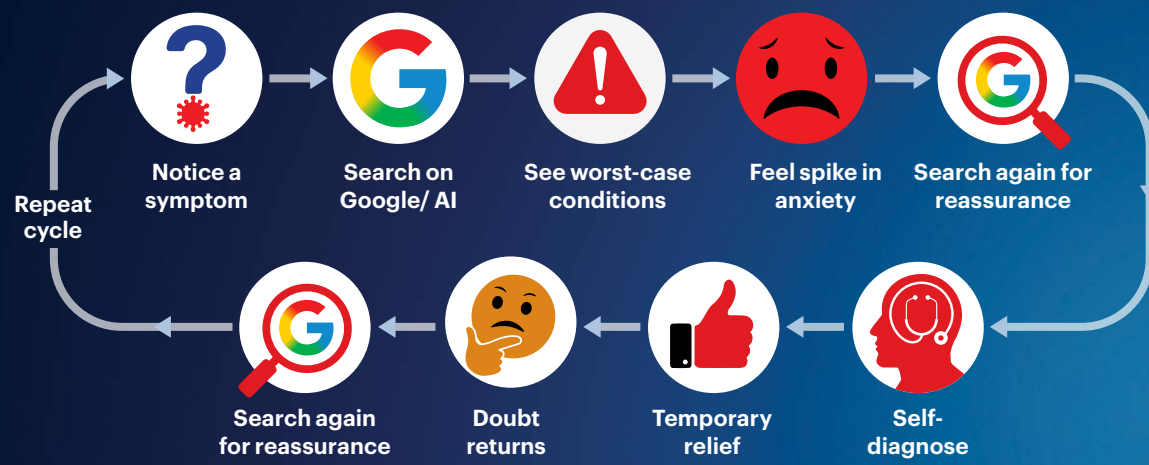
- Use AI for basic awareness, not diagnosis
- Cross-check with credible sources
- Note symptoms and prepare questions before consulting doctor

DON'Ts

- Self-diagnose serious conditions
- Start medication based on AI
- Rely on one search result
- Assume worst-case scenarios
- Delay seeing a doctor

ANXIETY ON A LOOP

Compulsive checking of symptoms can lead to misdiagnoses and abuse of medication as well as lower trust in the healthcare system and over-extension of hospital resources. Here is how the cycle goes:



or polycystic ovarian syndrome now rechristened Polyendocrine Metabolic Ovarian Syndrome (PMOS). Another patient, clutching earlier reports, is debating whether his chest discomfort is cardiac or gastric. None has been to the doctor yet.

Across Indian hospitals, this kind of patient is becoming a familiar sight. Department after department—from neurology and cardiology to gynaecology, oncology and psychiatry—reports patients coming to them with simple symptoms that they believe reflect serious disease. "Headaches, tingling, dizziness, or brief memory lapses, once considered minor or transient, are now often interpreted as serious conditions such as stroke, brain tumours, or neurological decline," says Dr Rishi Sharma, consultant-paediatric neurologist at Delhi's Rainbow Hospital. "As a result, many patients walk in with a diagnosis already in their heads, often accompanied by heightened anxiety."

Nowhere is cyberchondria more emotionally combustible than in oncology. "Cyberchondria is increasingly common among both young and older adults. Many now interpret fatigue, pain, unexplained weight loss, or lumps as cancer after an internet search," says Dr Siddharth Shankar Sood, senior consultant of oncology at Fortis Escorts Hospital, Faridabad.

Dr Meera Singh (*name changed on request*), who heads the internal medicine department at a national hospital chain, recalls a patient at her OPD who was convinced he needed tuberculosis medication urgently because his cousin had TB. He interpreted Singh's repeated advice for tests before treatment as deliberate denial of care. It took nearly an hour of her OPD time to finally persuade him. The TB test was negative, but so anxious was her patient that he repeated it twice before accepting the result. He later confessed an AI tool had warned him about TB exposure.

Orthopaedic specialists face similar cases. "There is a growing trend of patients with back pain, knee pain and posture-related concerns following internet advice on generic exercises or one-size-fits-all routines without understanding the root cause of their pain," says Dr Ramkinkar Jha, director of orthopaedics at the C.K. Birla Hospital in